

TRIME MR

Active Ingredient: Trimetazidine Dihydrochloride MR Tablets 35mg
Excipients: Anhydrous Calcium Hydrogen Phosphate, Colloidal Anhydrous Silica, Polyethylene Oxide, Povidone K-90, Xanthan Gum, Magnesium Stearate (synthetic), Opdary II Brown, Glycerol and Purified Water

Product Description
A pinkish brown coloured round biconvex film coated with “TZN 35” embossed on one side and plain on the other side.

Pharmacodynamics
ATC Code: C01EB15

Mechanism of Action
By preserving energy metabolism in cells exposed to hypoxia or ischaemia, trimetazidine prevents a decrease in intracellular ATP levels, thereby ensuring the proper functioning of ionic pumps and transmembrane sodium-potassium flow whilst maintaining cellular homeostasis. Trimetazidine inhibits β-oxidation of fatty acids by blocking long-chain 3-ketoacyl-CoA thiolase, which enhances glucose oxidation. In an ischaemic cell, energy obtained during glucose oxidation requires less oxygen consumption than in the β-oxidation process. Potentiation of glucose oxidation optimizes cellular energy processes, thereby maintaining proper energy metabolism during ischaemia.

Pharmacodynamic effects
In patients with ischaemic heart disease, trimetazidine acts as a metabolic agent, preserving the myocardial high-energy phosphate intracellular levels. Anti-ischemic effects are achieved without concomitant haemodynamic effects.

Pharmacokinetics
After oral administration, maximum concentration is found, on average, 5 hours after taking the tablet. Over 24 hours the plasma concentration remains at levels above or equal to 75% of the maximum concentration for 11 hours.

Steady state is reached by the 60th hour, at the latest.
The pharmacokinetic characteristics of trimetazidine 35mg are not influenced by meals.
The apparent distribution volume is 4.8 l/kg; protein binding is low. Trimetazidine is eliminated primarily in the urine, mainly in the unchanged form.

The elimination half-life of trimetazidine 35mg is an average of 7 hours, and 12 hours in those aged more than 65 years.
Total clearance of trimetazidine is the result of major renal clearance which is directly correlated to creatinine clearance and, to a lesser extent, to liver clearance which is reduced with age.

Special population
Elderly: The elderly may have increased trimetazidine exposure due to age-related decrease in renal function.
Renal Impairment: Trimetazidine exposure is increased on average by 1.7 in patients with moderate renal impairment (creatinine clearance between 30 and 60 ml/min) and on average by 3.1 fold in those with severe renal impairment (creatinine clearance below 30 ml/min). No safety concerns were observed in this population as compared with the general population.
Paediatric population: The pharmacokinetics of trimetazidine has not been established in the paediatric population (<18 years).

Indication
Trimetazidine is indicated in adults as add-on therapy for the symptomatic treatment of patients with stable angina pectoris who are inadequately controlled by or intolerant to first-line antianginal therapies.

Recommended Dose
Dosage
Route of Administration: Oral
The dose is one tablet of trimetazidine 35mg twice daily, i.e. once in the morning and once in the evening during meals.
The benefit of the treatment should be assessed after three months and trimetazidine should be discontinued if there is no treatment response.

Special populations
Patients with renal impairment: In patients with moderate renal impairment (creatinine clearance [30-60] mL/min) the recommended dose is one tablet of 35mg in the morning during breakfast.
Elderly patients: The elderly may have increased trimetazidine exposure due to age-related decrease in renal function. In patients with moderate renal impairment (creatinine clearance [30-60] mL/min), the recommended dose is one tablet of 35mg in the morning during breakfast. Dose titration in elderly patients should be exercised with caution.
Paediatric population: The safety and efficacy of trimetazidine in children aged below 18 years have not been established. No data are available

Contraindications
Hypersensitivity to the active substance or to any of the excipients listed. Parkinson disease, parkinsonian symptoms, tremors, restless leg syndrome and other related movement disorders
Severe renal impairment (creatinine clearance <30 mL/min)

Warnings and Precautions
This medicinal product is generally not recommended during breastfeeding.

Athletes: This medicinal product contains a drug substance that may give a positive result in antidoping tests.
This medicinal product is not a curative treatment for angina attacks, nor is it indicated as an initial treatment for unstable angina, nor myocardial infarction, nor in the pre-hospital phase nor during the first days of hospitalisation.

In the event of an angina attack, the coronaropathy should be re-evaluated and an adaptation of the treatment considered (medicinal treatment and possibly revascularisation).
Trimetazidine can cause or worsen parkinsonian symptoms (tremor, akinesia, hypertonia), which should be regularly investigated, especially in elderly patients. In doubtful cases, patients should be referred to a neurologist for appropriate investigations.
The occurrence of movement disorders such as parkinsonian symptoms, restless leg syndrome, tremors, gait instability should lead to definitive withdrawal of trimetazidine.
These cases have a low incidence and are usually reversible after treatment discontinuation. The majority of the patients recovered within 4 months after trimetazidine withdrawal. If parkinsonian symptoms persist more than 4 months after drug discontinuation, a neurologist opinion should be sought.
Falls may occur, related to gait instability or hypotension, in particular in patients taking antihypertensive treatment.



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Unit 3

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MEDREICH LIMITED						Title: Art Work Approval Form									
Product		Trime MR Tabs 35mg - S/L - Eng - PIL (SAP Descr.: TRIME MR 35MG TAB S/L ENG-PIL-WA-HEALOL)				Specification:		Printed on 40.5 (±3.5) GSM news print paper							
Customer		Healol Pharmaceuticals - Malaysia													
Reason for Issue		PIF No.: 1130PI2420009 New Product				Colours:		Single - Black							
Related FG Codes		130XXXX		Pharmacode No.		2966		Dimensions:		200 x 160 mm (Open Size) 200 x 25 mm (Folded Size)					
Item Code		1227640-V1 (WOA)		Blister Layout No.		NA		No. of Folds (only for PIL)		03		Artwork made to		100%	
				Carton Layout No.		NA									
				PIL Layout No.		NA									
Verified By						Approved By									
PDC:				PDC:		Regulatory:				CQA:					

Caution should be exercised when prescribing trimetazidine to patients in whom an increased exposure is expected:

- Moderate renal impairment
- Elderly patients older than 75 years old

Interactions with Other Medicaments
No drug interaction has been reported.

Pregnancy and Breastfeeding

Pregnancy

No data available. Direct or indirect harmful effects with respect to reproductive toxicity is not seen in available animal studies. As a precautionary measure, it is preferable to avoid the use of trimetazidine during pregnancy.

Breastfeeding

It is unknown whether trimetazidine/ metabolites are excreted in human milk. A risk to the new-borns/ infants cannot be excluded. Trimetazidine should not be used during breastfeeding.

Fertility

Available reproductive toxicity studies have shown no effect on fertility in female and male rats.

Side Effects

Adverse reactions are listed by system organ class and specific adverse experience term in the table below:

System Organ Class	Frequency	Preferred Term
Nervous system disorders	Common	Dizziness, headache
	Not known	Parkinsonian symptoms (tremor, akinesia, hypertonia), gait instability, restless leg syndrome, other related movement disorders, usually reversible after treatment discontinuation
	Not known	Sleep disorders (insomnia, drowsiness)
Ear and labyrinth disorders	Not known	Vertigo
Cardiac disorders	Rare	Palpitations, extrasystoles, tachycardia
Vascular disorders	Rare	Arterial hypotension, orthostatic hypotension that may be associated with malaise, dizziness or fall, in particular in patients taking antihypertensive treatment, flushing
Gastrointestinal disorders	Common	Abdominal pain, diarrhoea, dyspepsia, nausea and vomiting
	Not known	Constipation
Skin and subcutaneous tissue disorders	Common	Rash, pruritus, urticaria.
	Not known	Acute generalized exanthematus pustulosis (AGEP), angioedema
General disorders and administration conditions	Common	Asthenia
Blood and lymphatic system disorders	Not known	Agranulocytosis, Thrombocytopenia, Thrombocytopenic purpura
Hepatobiliary disorders	Not known	Hepatitis

Symptoms and Treatment of Overdose

Limited information is available on trimetazidine overdose. Treatment should be symptomatic.

Effect on Ability to Drive and Use Machine

Trimetazidine does not have haemodynamic effects, however cases of dizziness and drowsiness have been observed in post-marketing experience which may affect the ability to drive and operate machinery.

Instructions for use

The tablet must be swallowed whole. Do not chew or crush the tablet.

Storage Condition

Store below 30°C.

Shelf Life

36 months

Nature and contents of container

10 tablets are packed in one Alu-PVC/PE/PVDC clear Blister pack. 10 Alu-PVC/PE/PVDC clear Blister pack of each 10 tablets along with Package Insert packed in a printed carton.

Product Registration Holder :

Healol Pharmaceuticals Sdn. Bhd.
No. 74-3, Jalan Wangsa Delima 6,
KLSC Wangsa Maju,
53300 Kuala Lumpur
Malaysia.

Manufacturer:

Medreich Limited
Unit III, 4/3 Avalahalli Anjanapura Post, Kanakapura Road
Bengaluru – 560 062, India.

Date of revision: 21 December 2022

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